TWELVE CATEGORIES OF WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

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for the Australian Care Alliance

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INTRODUCTION

A careful examination of the evidence from those jurisdictions that have laws permitting either assisted suicide or euthanasia shows that at least twelve categories of people would be at risk of wrongful deaths.

Some proponents of legalising assisted suicide or euthanasia admit that it is the case that wrongful deaths will occur.

Henry Marsh, a noted British neurosurgeon and champion of assisted suicide, famously said,

"*Even if a few grannies are bullied into committing suicide, isn't that a price worth paying so that all these other people can die with dignity?*"

This is the question that anyone considering this issue needs to ask.

The proper tests for a law permitting assisted suicide or euthanasia are the ones that are usually applied to any proposal to reintroduce capital punishment:

"Can we craft a law that will ensure there will not be even one wrongful death?"

"Can we ensure that any deaths under this law are humane - that is both rapid and peaceful?"

Both simple logic and the available evidence show that neither of these outcomes are achievable.

This publication is kept up to date and is readily available online for consultation or downloading at:

https://www.australiancarealliance.org.au/flawed_experiments



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WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA
WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

A WRONG DIAGNOSIS

If a person dies by assisted suicide or euthanasia following **a mistaken diagnosis that the person has a terminal illness** then that is a wrongful death – with no remedy.



It was only after the family of retired Italian magistrate Pietro D'Amico, aged 62, insisted on an autopsy that he was found not to have a terminal illness at all, despite being given such a diagnosis by both Italian and Swiss doctors prior to undergoing assisted suicide in Switzerland.[1]

Peter Sefton-Williams, aged 71, began planning his assisted suicide with Dignitas in Switzerland after two specialists diagnosed him with motor neurone disease. He was told not to make any plans beyond six months. A few months later further tests showed that he had been misdiagnosed. He did not have motor neurone disease but Multifocal Motor Neuropathy, a mild condition that is not terminal and which, in most cases, is treatable.[2]



[1] https://www.thelocal.ch/20130711/assisted-suicide-in-question-after-botched-diagnosis

[2] https://www.spectator.co.uk/article/my-false-diagnosis-exposes-a-key-flaw-in-the-assisted-dying-bill/

According to evidence given by Dr Stephen Child, Chair of the New Zealand Medical Association to the New Zealand parliamentary inquiry into the practice of euthanasia:

"*On diagnosis, 10 to 15 per cent of autopsies show that the diagnosis was incorrect. Three per cent of diagnoses of cancer are incorrect*".

Dr Child said this scope for error was too large, when weighed against the outcome. "*This is an irreversible decision in which the consequences are final*." [3]

Ten per cent of cases in Australia are misdiagnosed according to Peter McClennan, chief executive at Best Doctors.[4]

An August 2018 report on Missouri resident Pasquale Michael Fatino, aged 52, who is suing three doctors at his former primary care clinic for a misdiagnosis of terminal cancer that caused him and his family unnecessary pain and suffering last year, illustrates that such mistakes do happen.[5]

Simply having two doctors diagnose that a person has a terminal illness is an illusory safeguard. There is no remedy for a wrongful death by assisted suicide based on misdiagnosis.

How many wrongful deaths from assisted suicide following misdiagnosis of a terminal illness are too many?

[3] http://www.stuff.co.nz/national/politics/84252580/euthanasia-toofinal-when-the-risk-of-error-is-to-great--doctors

[4] https://amp.afr.com/business/insurance/insurance-companies/mlc-life-to-expand-best-doctors-service-20170827-gy4zfk

[5] https://www.kansascity.com/news/business/health-care/article216764080.html

A WRONG PROGNOSIS

If a person dies by assisted suicide or euthanasia **after being told in error that they have less than six months to live when they may have many years of life ahead of them** then that is a wrongful death – with no remedy.

Medical literature on errors and uncertainty in prognosis

A survey of the medical literature on prognosis indicates that **an accurate prognosis is notoriously difficult to make**.

A study on the accuracy of prognoses in oncology found that "*discrimination between patients who would survive for one year and those who would not was very poor*".[6]

Another paper describes doctors' ability at predicting survival at 1 year as "*only slightly better than a random guess*".[7]

Australia's *National Consensus Statement: essential elements for safe and high-quality end-of-life care* wisely observes: (on page 17)

> *Predicting prognosis and the timing of dying can be difficult. For some patients, it may be difficult to distinguish clinical deterioration that is reversible from deterioration that is irreversible and part of the normal dying process. In such cases, it may be appropriate to consider a trial of treatment for a defined period to assess reversibility of a patient's deterioration*.[8]

One recent study of prognostic accuracy for brain cancer found that:

> *All physicians had individual patient survival predictions that were incorrect by as much as 12-18 months, and 14 of 18 physicians had individual predictions that were in error by more than 18 months. Of the 2700 predictions, 1226 (45%) were off by more than 6 months and 488 (18%) were off by more than 12 months.*" Of particular relevance to the use of a prognosis of expected death to grant access to assisted suicide is the finding that "*In this study all physicians were unable to accurately predict longer term survivors. Despite valuable clinical data and predictive scoring techniques, brain and systemic management often led to patient survivals well beyond estimated survivals*.[9]

[6] https://www.sciencedirect.com/science/article/pii/S0895435696003162

[7] http://journals.plos.org/plosone/article?id=10.1371/journal.pone.0161407

[8] https://www.safetyandquality.gov.au/wp-content/uploads/2015/05/National-Consensus-Statement-Essential-Elements-forsafe-high-quality-end-of-life-care.pdf

[9] https://www.ncbi.nlm.nih.gov/pubmed/24160479

A study published in 2000 in the British Medical Journal found that physicians only made accurate (within 33% margin either way) prognoses in 20% of cases for terminally ill patients.

Significantly for the use of a prognosis in allowing access to assisted suicide or euthanasia is the finding that in 17% of cases physicians were overly pessimistic in their prognosis by more than 33% and out by a factor of 2 in 11.3% of cases.[10]

In other words, perhaps **more than one in ten people given a prognosis of 12 months to live may live for 2 years or more**.

Evidence from Oregon and Washington

Evidence from the states of Oregon and Washington, where assisted suicide is legal, demonstrates conclusively that physicians make significant errors in determining a prognosis of less than six months to live in the context of a request to access assisted suicide.

Oregon's *Death With Dignity Act* provides that before prescribing a lethal substance a doctor must first determine whether a person has a "terminal disease". This is defined by section 127.800 (12) of the Oregon Revised Statute to mean "*an incurable and irreversible disease that has been medically confirmed and will, within reasonable medical judgment, produce death within six months*".

In 2018 one person ingested lethal medication 807 days (2 years 2 ½ months) after the initial request for the lethal prescription was made. The longest duration between initial request and ingestion recorded is 1009 days (that is 2 years and 9 months).[11] Evidently in these cases the prognosis was wildly inaccurate.

Washington State's *Death With Dignity Act*, based on Oregon's, came into operation on 9 March 2009.

Although this Act specifies that only persons with "*six months or less to live*" may request lethal doses of medication from a physician, the data shows that in each year between 5% and 17% of those who die after requesting a lethal dose do so more than 25 weeks later with one person in 2012 dying nearly 3 years (150 weeks) later, one person in 2015 dying nearly two years later (95 weeks), one person in 2016 dying more than two years (112 weeks) later and one person in 2017 dying 1 year and 7 months later (81 weeks).[12]

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[10] http://www.bmj.com/content/bmj/320/7233/469.full.pdf

[11] Oregon Public Health Division, *Oregon Death With Dignity Act: 2018 Data Summary, Table 1,* p.13 https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year21.pdf

[12] https://www.doh.wa.gov/YouandYourFamily/IllnessandDisease/DeathwithDignityAct/DeathwithDignityData

Evidently in each of these cases the prognosis was wildly inaccurate.

And of course **we will never know how long many of those who take the lethal dose within the 6 month period might have lived** if they hadn't been given assisted suicide.



The example of Jeanette Hall, who is still alive today after commencing the process of seeking assisted suicide in Oregon in 2000, illustrates the danger of making assisted suicide available to people when first diagnosed with a terminal illness. Thankfully for Jeanette her doctor refused to collaborate in assisting her suicide and helped her find hope – and effective treatment – instead.[13]

There is always a better way than the counsel of despair offered by proponents of assisted suicide.

How many wrongful deaths from assisted suicide following a mistaken prognosis are too many?

[13] https://www.dailysignal.com/2015/05/18/assisted-suicide-how-one-woman-chose-to-die-then-survived/

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

UNAWARE OF AVAILABLE TREATMENT

Will patients suffer wrongful deaths?

Some assisted suicide or euthanasia laws purport to provide an additional safeguard by requiring at least one doctor with relevant specialist experience to assess the person and inform them of all relevant information about the person's condition.

However, despite such provisions the evidence from jurisdictions which have legalised assisted suicide or euthanasia shows that **some people miss out on treatment that could have helped them and instead suffer a wrongful death** by assisted suicide or euthanasia.

Northern Territory

The *Rights of the Terminally Ill Act 1995* (the ROTI Act) was in operation in the Northern Territory from 1 July 1996 until it was suppressed by the Commonwealth's *Euthanasia Laws Act 1997* on 27 March 1997.

During the nine month period in which the ROTI Act was in effect and under its provisions, four people were assisted to terminate their lives by Dr Philip Nitschke.

Case studies on these four deaths have been published.[14] The principal author of this paper is Professor David Kissane, who is a consultant psychiatrist and professor of palliative medicine. Philip Nitschke is a co-author of the paper.

The case studies examine how the conditions required by the ROTI Act were met. Cases numbered 3, 4, 5 and 6 in this paper refer to those cases which ended with the person's life being terminated with the assistance of Dr Philip Nitschke.

The ROTI Act provided that a "*medical practitioner who receives a request*" may, if certain conditions are met, "*assist the patient to terminate the patient's life*".

The conditions to be met included that a second "*medical practitioner who holds prescribed qualifications, or has prescribed experience, in the treatment of the terminal illness from which the patient is suffering*" has examined the patient and has confirmed *"(A)*

14 http://www.healthprofessionalssayno.info/uploads/1/0/9/2/109258189/seven_deaths_in_darwin_case_studies_unde.pdf

the first medical practitioner's opinion as to the existence and seriousness of the illness; (B) that the patient is likely to die as a result of the illness; and (C) the first medical practitioner's prognosis" (Section 7(1)(c)(i) and (iii)).

In case 4, there was no consensus that the person was terminally ill. The person was diagnosed with mycosis fungoides. "*One oncologist gave the patient's prognosis as 9 months, but a dermatologist and a local oncologist judged that she was not terminally ill. Other practitioners declined to give an opinion. In the end an orthopaedic surgeon certified that the ROTI provisions for terminal illness had been complied with*."

In case 3 the patient may have benefited from radiotherapy or strontium but neither of these was available in the Northern Territory.

In case 5, the patient had an obstruction and was clinically jaundiced. The ROTI Act required Dr Nitschke as a "*medical practitioner who receives a request*" to have "*informed the patient of the nature of the illness and its likely course, and the medical treatment, including palliative care, counselling and psychiatric support and extraordinary measures for keeping the patient alive, that might be available to the patient*." However, Kissane reports that "*when questioned about options like stenting for obstructive jaundice or the management of bowel obstruction*" Dr Nitschke "*acknowledged limited experience, not having been involved in the care for the dying before becoming involved with the ROTI Act*."

This raises doubts as to whether the patient in this case – who was reported by Dr Nitschke to exhibit "*indecisiveness*" over a two month period about whether or not to request euthanasia – would still have done so if he had been given better symptomatic relief for the jaundice and obstruction.

Netherlands

The review committees in the Netherlands are required to consider whether all the conditions of the euthanasia law have been met in each case. The law there requires assessment by a relevant specialist before a person is euthanased.

In case 15 of the 2011 annual report the Regional Euthanasia Review Committees conclude that the attending physician failed to achieve an accurate diagnosis of the woman's back pain and only prescribed limited pain relief medication. Consequently it could not be said that the woman's pain was definitively unrelievable. Of course the woman can get no relief from this finding of error on the part of the doctor who failed her and then euthanased her as she is already dead by euthanasia.[15]

The same lack of remedy applies to the two cases of people with dementia who were euthanased in 2012 in relation to which the Review Committees found "*not to have been handled with due care*".[16]

[15] http://www.euthanasiecommissie.nl/Images/RTE.JV2011.ENGELS.DEF_tcm52-33587.PDF
[16] http://www.euthanasiecommissie.nl/Images/JV.RTE2012.engelsDEF2_tcm52-39100.pdf

In Case 2015-01 the Review Committee found a lack of due care before euthanasia was carried out on a woman with a history of stomach pains from an undiagnosed cause, who was reluctant to be examined by a geriatrician. No further action was taken on this case.[17]

Canada

A study of 45 people who were euthanased at Ottawa Hospital for lung cancer showed that in 13 cases (28.9%) there was no confirmation of the condition by biopsy - an otherwise standard diagnostic procedure. In 10 cases (22%) there was no consultation at any stage with a medical oncologist to discuss prognosis and treatment options.

The time between diagnosis and euthanasia being performed was as short as 3 weeks.

Treatments for lung cancer have improved considerably over the last few years leading to a reduction in mortality and an increase in the five year survival rate which is now 21.7%, according to the (US) National Cancer Institute.

This study confirms that when euthanasia is legalised it will inevitably result in premature an unnecessary deaths of people who will miss out on effective, helpful treatment. In some cases this will be due to a failure of the medical practitioners assessing them for access to euthanasia to have the specialist knowledge to properly and comprehensively advise the person on the most recent treatment options.

In other cases a person may adamantly request euthanasia when first diagnosed with cancer and be granted it quickly (as this study shows as soon as 3 weeks after diagnosis). This does not allow any time for the person to process the new situation, to receive treatment for clinical depression or to be given time to reflect and consider treatment options.

The study was carried out by Dr Sara Moore and the results presented on 9 September 2021 at the 2021 World Conference on Lung Cancer.

Dr Moore said

> *Biomarker-driven targeted therapy and immunotherapy offer effective and tolerable new treatments, but a subset of patients undergo medical assistance in dying [euthanasia] without accessing -- or, in some cases, without being assessed for -- these treatment options.*

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[17] https://www.nvve.nl/files/8414/6166/0719/RTE_jaarverslag2015DEF.pdf

Given the growing number of efficacious and well-tolerated treatment options in lung cancer, consultation with an oncologist may be reasonable to consider for all patients with lung cancer who request medical assistance in dying.

Commenting on the presentation, geriatric and palliative care physician, Dr Monica Malec observed that:

> *Patients are seeking this option [euthanasia] despite the availability of more effective and more tolerable treatment options.*
>
> *Existing literature suggests that loss of autonomy, control, and dignity are the primary drivers for seeking medical assistance in dying rather than uncontrolled symptoms, and the decision to pursue medical assistance in dying [euthanasia] may occur pre-illness.*

Conclusion

No legal framework permitting assisted suicide or euthanasia has yet been established which effectively ensures that all persons being killed or helped to commit suicide are accurately diagnosed and properly informed about all treatment options available for their condition.

People are being killed or helped to commit suicide who could have benefited from treatment

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

NO ACCESS TO PALLIATIVE CARE

Is assisted suicide needed to deal with physical pain?

There is a telling disconnect between the focus of assisted suicide and euthanasia laws **when they are being proposed** and **after they have been implemented**.

During the proposal phase the focus is almost universally on an alleged group of hard cases, few in number, who, it is said, are suffering unbearable physical pain or other physical symptoms that cannot be relieved by even the best palliative care. This claim is based largely on anecdotal evidence, often from earlier decades before recent improvements in palliative care.

- Proponents of assisted suicide laws falsely claim it is needed to relieve physical pain in a small number of hard cases
- Palliative care experts say that pain can be alleviated, including by the use of palliative sedation in some cases
- Physical pain is not among the main reasons people in Oregon give for seeking assisted suicide; loss of autonomy and being a burden on others are more common concerns
- Ian Haines is a medical oncologist who has changed his mind on euthanasia due to recent developments in palliative care
- The Australian Pain Management Association says legalising assisted suicide "*may lead to government having an opportunity for people to end their life with the help of another person rather than investing in early pain management support and the medical treatment and community support that people need in order to have a 'good death' and die with dignity*"

Assisted suicide for pain management?

After implementation it becomes clearer that the real focus is on autonomy – an alleged right to assistance to die at a time of one's own choosing for any reason.

Proposals to legalise assisted suicide for a select group of people often include a criterion such as "*suffering from a serious and incurable condition which is causing enduring and unbearable suffering that cannot be relieved in a manner the patient deems tolerable*".

Proponents often claim that palliative care cannot relieve all pain.

For example the Victorian parliamentary committee report *End of Life Choices* falsely claimed (p. 206) that palliative care specialists gave evidence that "*not all pain can be alleviated*".[18] Footnote 809 references "*Palliative Care Victoria, Submission, pp. 14–15*". This is most misleading as what that submission actually states is that in the rare cases when all other methods of palliation for pain and other symptoms fail, palliative sedation therapy is available to provide adequate relief of suffering.[19]

Palliative Care Victoria: pain can be alleviated

The Palliative Care Victoria submission relevantly states:

Achieving the effective management of pain and other symptoms is a high priority in the care of people with a life limiting illness and people who are dying. Where these symptoms are not readily alleviated by general health and care services, a referral to access the specialised expertise of palliative care services should be made. In most cases, specialist palliative care teams are able to address the person's physical pain and other symptoms and to respond to their psycho-social, emotional, spiritual and cultural needs so that they are able to live and die well with dignity. However, a small minority of patients experience refractory symptoms such as agitated delirium, difficulties breathing, pain and convulsions.

Refractory symptoms are defined as:

"pain or other symptoms for which all possible treatment has failed, or it is estimated that no methods are available for palliation within the time frame and the risk-benefit ratio that the patient can tolerate."

A patient with refractory (unrelieved) distress "must have received skilled multidimensional management directed at the physical, psychological and existential dimensions of the symptom before a symptom is considered refractory." Prudent application of palliative sedation therapy may be used in the care of selected palliative care patients with otherwise refractory distress.

[T]he level of sedation used should be the lowest necessary to provide adequate relief of suffering: "The doses of medications should be increased or reduced gradually to a level at which suffering is palliated with a minimum suppression of the consciousness levels and undesirable effects, with documentation of the reason for changes and response to such manoeuvres." Only under exceptional circumstances is deep and continuous sedation required from initiation of palliative sedation therapy.

Having misleadingly claimed that palliative care experts concede that pain and other physical symptoms cannot be alleviated and that therefore some Victorians are faced

[18] http://www.parliament.vic.gov.au/images/stories/committees/SCLSI/EOL_Report/LSIC_58-05_Text_WEB.pdf

[19] http://www.parliament.vic.gov.au/images/stories/committees/lsic/Submissions/Submission_236_-_Palliative_Care_Victoria.pdf

with unavoidably painful deaths the End of Life Choices majority report then niftily substituted **a purely subjective notion of suffering** for that of objective pain and other physical symptoms.

Assisted suicide in Oregon: *not* for pain

"*Enduring and unbearable suffering*" is a phrase capable of including the prime reasons given by the 1275 people who have died under Oregon's assisted suicide law:

- A steady loss of autonomy (90.9%)
- Less able to engage in activities making life enjoyable (89.5%)
- Loss of dignity (75.7%)
- Loss of control of bodily functions, such as incontinence and vomiting (45.7%)
- Physical or emotional burden on family, friends, or caregivers (43.7%) .[20]

Only 327 out of 1275 people (25.8%) mentioned inadequate pain control or concern about it. Earlier annual reports noted that "*Patients discussing concern about inadequate pain control with their physicians were not necessarily experiencing pain.*"[21]

In 2017 just over one in five (21%) mentioned concern about inadequate pain control but more than half (55.25%) cited concerns about being a burden on others as a reason for seeking assisted suicide.[22]

Ian Haines: a change of mind on euthanasia



Associate Professor Ian Haines, MBBS is a medical oncologist and palliative medicine specialist. He comments:

As an oncologist with 35 years' full-time experience, I have seen palliative care reach the point where the terminally ill can die with equal or more dignity than euthanasia will provide. It is now very effective and increasingly available for two of the three possible ways of dying, outside of sudden unexpected death, which are advanced cancer and chronic relapsing and remitting organ-specific disease such as heart or lung failure.

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20 https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year20.pdf

21 http://public.health.oregon.gov/ProviderPartnerResources/EvaluationResearch/DeathwithDignityAct/Documents/year6.pdf

22 https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year20.pdf

Palliative care is also available for people with chronic progressive cognitive diseases such as dementia. This is the fourth way of dying and perhaps the most feared of all.

Like Andrew Denton and others who have observed unbearable suffering in loved ones and the terrible failures of modern medicine in the past, I had once believed that euthanasia was the only humane solution.

I no longer believe that.

If the Victorian government legalises assisted dying for people suffering from serious and incurable conditions, it will be the wrong choice. It is not necessary and, as outlined in the minority report to government, it will inevitably increase the pressure, both stated and perceived, for some chronically ill patients to move on and stop being a burden.[23]

Australian Pain Management Association

The Australian Pain Management Association is concerned legalising assisted suicide "*may lead to government having an opportunity for people to end their life with the help of another person rather than investing in early pain management support and the medical treatment and community support that people need in order to have a 'good death' and die with dignity*".

"*Australia has achieved a high level of expertise in pain and palliative medicine including well-coordinated multidisciplinary care. However, this needs to be delivered across the health care system, particularly at home or in residential aged care facilities. Less than optimal analgesia and symptom control are major obstacles to quality end-of-life care.*

"[E]ndorsing such a Bill [may] let health services off the hook from providing the best and widest pain management services."[24]

Conclusion

No case for legalising assisted suicide can properly be made on the basis that this is the only possible response to people facing unrelievable pain.

Every Australian deserves access to gold standard palliative care which can alleviate pain, including using palliative sedation as a last resort.

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[23] http://www.theage.com.au/comment/i-believed-that-euthanasia-was-the-only-humane-solution-i-no-longer-believe-that-20161118-gss921.html

[24] https://www.painmanagement.org.au/

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

DENIED FUNDING FOR MEDICAL TREATMENT

People who are denied funding for medical treatment by medical insurers or the public health system but are offered funding for assisted suicide or euthanasia, as has happened in Oregon, California and Canada are at risk of wrongful deaths either by being denied needed treatment or bullied into agreeing to assisted suicide.

Oregon

In Oregon, of those who died from ingesting a lethal dose of medication in 2017, more than one in twenty (5.6%) mentioned the *"financial implications of treatment"* as a consideration.[25] While this percentage is relatively small it is appalling that since 1998 forty seven (47) Oregonians have died from a lethal prescription after expressing concerns about the financial implications of treatment.

In two notorious cases, those of Barbara Wagner[26] and Randy Stroup,[27] the Oregon Health Plan informed a patient by letter that the particular cancer treatment recommended by their physicians was not covered by the Plan but that the cost of a lethal prescription to end their life would be covered.



The news from Barbara Wagner's doctor was bad, but the rejection letter from her insurance company was crushing.

The 64-year-old Oregon woman, whose lung cancer had been in remission, learned the disease had returned and would likely kill her. Her last hope was a $4,000-a-month drug that her doctor prescribed for her, but **the insurance company refused to pay**.

[25] http://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year20.pdf

[26] https://abcnews.go.com/Health/story?id=5517492

[27] http://www.foxnews.com/story/2008/07/28/oregon-offers-terminal-patients-doctor-assisted-suicide-instead-medical-care.html

What the Oregon Health Plan did agree to cover were drugs for a physician-assisted death. Those drugs would cost about $50.

"It was horrible," Barbara Wagner explains "I got a letter in the mail that basically said if you want to take the pills, we will help you get that from the doctor and we will stand there and watch you die. But we won't give you the medication to live."[28]



Since the spread of his prostate cancer, 53-year-old Randy Stroup of Dexter, Oregon, has been in a fight for his life. Uninsured and unable to pay for expensive chemotherapy, he applied to Oregon's state-run health plan for help.

Lane Individual Practice Association (LIPA), which administers the Oregon Health Plan in Lane County, responded to Stroup's request with a letter saying **the state would not cover Stroup's pricey treatment, but would pay for the cost of physician-assisted suicide**.

"It dropped my chin to the floor," Stroup said. "[How could they] not pay for medication that would help my life, and yet offer to pay to end my life?"[29]

California



Stephanie Packer, a wife and mother of four who was diagnosed with a terminal form of scleroderma, said her insurance company initially indicated it would pay for her to switch to a different chemotherapy drug at the recommendation of her doctors.

But shortly after California's End of Life Option Act, which authorizes physicians to diagnose a life-ending dose of medication to patients with a prognosis of six months or less to live, went into effect, Ms. Packer's insurance company had a change of heart.

[28] https://www.youtube.com/watch?v=erzYKNrsx_I

[29] https://www.youtube.com/watch?v=fKOT3oujULI

“And when the law was passed, it was a week later I received a letter in the mail saying they were going to deny coverage for the chemotherapy that we were asking for,” Ms. Packer said.

She said she called her insurance company to find out why her coverage had been denied. On the call, she also asked whether suicide pills were covered under her plan.

“And she says, ‘Yes, we do provide that to our patients, and you would only have to pay $1.20 for the medication,’” Ms. Packer said.

Ms. Packer said her doctors have appealed the insurance company’s decision twice, to no avail. She said **the assisted-suicide law creates an incentive for insurance companies to deny terminally ill patients coverage**.[30]



Dr Brian Callister, associate professor of internal medicine at the University of Nevada, said he tried to transfer two patients to California and Oregon for procedures not performed at his hospital. Representatives from two different insurance companies denied those transfer requests by phone, he said.

The patients were not terminal, but “would have become terminal without the procedures.”

And in both cases, the insurance medical director said to me, "Brian, we’re not going to cover that procedure or the transfer, but would you consider assisted suicide?’ ”

The phone calls took place last year within the span of a month, Dr. Callister said. He said he did nothing to prompt the suggestion in either case.[31]

Canada



Roger Foley, who has a crippling brain disease, has been seeking support to live at home. He is currently in an Ontario hospital that is threatening to start charging him $1,800 a day. The hospital has told Roger that his other option is euthanasia or assisted suicide under Canada’s medical assistance in dying law.[32]

[30] https://www.youtube.com/watch?v=1NoNthOhMd4

[31] https://youtu.be/JzafMM9QCAg

[32] https://www.australiancarealliance.org.au/canada_assisted_suicide_not_assisted_living

Belgium

Christophe, a father of four young children aged 4, 6, 7 and 10 years old, has begun the process of seeking euthanasia in Belgium, after the Belgian health system refused funding support for the only available treatment for the rare blood disease - paroxysmal nocturnal hemoglobinuria – which he has suffered from since he was 17 years old.

The disease which is caused by a random mutation and is not inheritable. It causes red blood cells to be attacked resulting in fatigue, a risk of thrombosis and organ deterioration. The disease is degenerative with no known cure. However, treatment with a monoclonal antibody called Soliris has proved successful in achieving significantly reduced hemolysis (destruction of red blood cells) leading to to an improvement in symptoms and a reduction of thrombotic events, which can prove fatal.

Six months treatment costs 50,000 euros.

"It would not save me but it could make me live a decade more with my children," says the young man. "What really makes me sick is that I know they're going to lose me."



Today, Christophe lives alone. He separated from the mother of his children two years ago, "to give her back her freedom," he says. "I was not too present, I slept all the time, it was always the hospitals ... It's tiring for a girl like her," he says.

Christophe feels so weak that he does not come out of his house anymore. "Every time I went out I grabbed a microbe," he says. "I'm at home, I do tidying up, I watch TV, I'm eating," he continues. His children visit him on weekends, always in the presence of his parents. "If I fall into a coma, it's not my 4-year-old daughter, my 10-year-old son, the youngest 7 and 6 years old, who can call an ambulance," he says.

Apart from these visits, Christophe no longer enjoys anything. "I live like a prison," he laments. He began thinking about euthanasia three years ago. Over the last year, he has embarked on the steps that would allow him to be euthanased.

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

MENTALLY ILL AT RISK

- People with a mental illness are at risk of wrongful death under any law authorising assisted suicide or euthanasia.
- In the Netherlands and Belgium mental illness is seen as a condition for which euthanasia or assisted suicide is increasingly considered to be an appropriate response.
- In Oregon and Washington State where the laws provide for *optional referral* for psychiatric assessment the evidence shows that the gatekeeping medical practitioners very seldom refer and that this results in persons with *treatable clinical depression* being wrongfully assisted to commit suicide.
- In the Northern Territory, where euthanasia was legal from July 1996-March 1997, and compulsory screening by a psychiatrist was required, there was a failure to adequately identify depression, demoralization or other psychiatric issues which may have been treatable in all four cases of persons killed under that regime.

Belgium

Belgium offers assisted suicide or euthanasia for persons suffering with depression or other mental illnesses.

A total of 201 people with psychiatric disorders were killed by euthanasia in Belgium between 2014 and 2017 including for mood disorders such as depression, bipolar disorder (73 cases); organic mental disorders, including dementia and Alzheimer's (60 cases); personality and behavioural disorders (23 cases); neurotic disorders, and disorders related to stressors including posttraumatic stress disorder (16 cases); schizophrenia and psychotic disorders (11 cases); organic mental disorders, including autism (10 cases) and complex cases involving a combination of several categories (8 cases).

Of these 201 cases there were 25 cases of people under 40 being killed by euthanasia. In relation to these troubled young people the Commission observes:

> *In the group of patients under 40, it is mainly personality and behavioral disorders. All these patients have been treated for many years, both outpatient and residential. There has always been talk of intractable suffering. For this type of disorder, serious*

psychological trauma at a very young age has been mentioned several times, such as domestic violence, psychological neglect or sexual abuse.[33]

Belgium is treating the victims of child abuse by domestic violence, neglect and sexual abuse by killing them.

US psychiatrist Dr Mark Komrad explains how profoundly the law on euthanasia has affected the practice of psychiatry in Belgium:

The criteria for euthanasia— a condition that is "insufferable and untreatable"— has called into existence a new category for the mentally ill who have those characteristics. As in the US, the notion of [a] truly "untreatable" condition in psychiatry really didn't exist in the Benelux countries, until their legislatures conjured that category into legal existence, thinking of the terminal somatic conditions with which physician administered euthanasia originally began. Once this category opened to "psychological suffering" it became a beckoning space which influenced how psychiatrists and their patients began to see some cases.[34]

Netherlands

Euthanasia is legally permitted in the Netherlands for dementia patients and for persons with depression or other mental health issues in the complete absence of any physical illness or suffering.

In 2017 there were 83 notifications of euthanasia or assisted suicide involving patients with psychiatric disorders. There were 166 notifications involving dementia. All these cases were in the absence of any other condition justifying euthanasia. Three of the dementia cases of euthanasia were performed on the basis of an advanced directive rather than a contemporary request by the person who was euthanased.[35]

Psychiatric conditions for which euthanasia was performed in 2015 included personality disorder with posttraumatic stress disorder and self-mutilation; and obsessive compulsive disorder.[36]

[33] https://organesdeconcertation.sante.belgique.be/sites/default/files/documents/8_rapport-euthanasie_2016-2017-fr.pdf

[34] http://www.psychiatrictimes.com/couch-crisis/psychiatrist-visits-belgium-epicenter-psychiatric-euthanasia

[35] https://www.euthanasiecommissie.nl/actueel/nieuws/2018/maart/7/vijftien-jaar-euthanasiewet-belangrijkste-cijfers-2017

[36] https://www.nvve.nl/files/8414/6166/0719/RTE_jaarverslag2015DEF.pdf

Oregon

Oregon's *Death With Dignity Act*, which has been operative since 1997, provides for medical practitioners to provide prescriptions for lethal medications to be taken later by the person for whom the lethal dose is prescribed.

Research by Linda Ganzini et al. found that "*Among terminally ill Oregonians who participated in our study and received a prescription for a lethal drug, one in six had clinical depression*".[37]

Depression is supposed to be screened for under the Act. However, of the 1275 people who have died by ingesting lethal dose prescribed under the Act only 62 (4.9%) were first referred for psychiatric evaluation.[38]

This means **as many as 150 people with depression may have been helped to commit suicide** without any such referral.

In 2011 Dr. Charles J. Bentz of the Division of General Medicine and Geriatrics at Oregon Health & Sciences University explained that Oregon's physician-assisted suicide law is not working well. He cited the example of a 76-year-old patient he referred to a cancer specialist for evaluation and therapy. The patient was a keen hiker and as he underwent therapy, he became depressed partly because he was less able to engage in hiking.

He expressed a wish for assisted suicide to the cancer specialist, who rather than making any effort to deal with the patient's depression, proceeded to act on this request by asking Dr Bentz to be the second concurring physician to the patient's request.

When Dr Bentz declined and proposed that instead the patient's depression should be addressed the cancer specialist simply found a more compliant doctor for a second opinion.

Two weeks later the patient was dead from a lethal overdose prescribed under the Act.

Dr Bentz concludes;

"*In most jurisdictions, suicidal ideation is interpreted as a cry for help. In Oregon, the only help my patient got was a lethal prescription intended to kill him.*" He urges other jurisdictions "*Don't make Oregon's mistake*."[39]

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[37] http://www.bmj.com/highwire/filestream/384131/field_highwire_article_pdf/0.pdf

[38] https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year20.pdf

Northern Territory

The *Rights of the Terminally Ill Act 1995* (the ROTI Act) was in operation in the Northern Territory from 1 July 1996 until it was suppressed by the Commonwealth's *Euthanasia Laws Act 1997* on 27 March 1997.

During the nine month period in which the ROTI Act was in effect and under its provisions, four people were assisted to terminate their lives by Dr Philip Nitschke.

Case studies on these four deaths have been published.[40] The principal author of this paper is Professor David Kissane, who is a consultant psychiatrist and professor of palliative medicine. Philip Nitschke is a co-author of the paper.

The case studies examine how the conditions required by the ROTI Act were met. Cases numbered 3, 4, 5 and 6 in this paper refer to those cases which ended with the person's life being terminated with the assistance of Dr Philip Nitschke.

From the case histories, it is apparent that cases 3 and 4 each had depressive symptoms.

In case 3, the patient had received "*counselling and anti-depressant medication for several years*". He spoke of feeling sometimes so suicidal that "*if he had a gun he would have used it*". He had outbursts in which he would "*yell and scream, as intolerant as hell*" and he "*wept frequently*".

Neither the patient's adult sons nor the members of the community palliative care team who were caring for him were told he was being assessed for euthanasia. "*A psychiatrist from another state certified that no treatable clinical depression was present*."

In case 4, "*the psychiatrist noted that the patient showed reduced reactivity to her surroundings, lowered mood, hopelessness, resignation about her future, and a desire to die. He judged her depression consistent with her medical condition, adding that side-effects of her antidepressant medication, dozepin, may limit further increase in dose*."

Kissane comments that "*case 4 was receiving treatment for depression, but no consideration was given to the efficacy of dose, change of medication, or psychotherapeutic management*." While Dr Nitschke "*judged this patient as unlikely to respond to further treatment*", Kissane, comments that "*nonetheless, continued psychiatric care seemed warranted – a psychiatrist can have an active therapeutic role in ameliorating suffering rather than being used only as a gatekeeper to euthanasia*".

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[39] http://blogs.theprovince.com/2011/12/05/province-letters-icbc-egypt-assisted-suicide-oregon-christmas-pre-marital-sex/

[40] http://www.healthprofessionalssayno.info/uploads/1/0/9/2/109258189/seven_deaths_in_darwin_case_studies_unde.pdf

In case 6 a key factor seemed to be the patient's distress at "*having witnessed*" the death of her sister who also had breast cancer, "*particularly the indignity of double incontinence*". She "*feared she would die in a similar manner*". She "*was also concerned about being a burden to her children, although her daughters were trained nurses*".

Kissane noted that "*fatigue, frailty, depression and other symptoms*" – not pain – were the prominent concerns of those who received euthanasia. He observed that "*palliative care facilities were underdeveloped in the Northern Territory, and patients in our study needed palliative care… There is a need to respond creatively to social isolation, and to treat actively all symptoms with early and skilled palliative care*."

Further concerns are raised by the report on case 5. Dr Nitschke reported that "*on this occasion the psychiatrist phoned within 20 min, saying that this case was straightforward*". This assessment took place on the day on which euthanasia was planned. This case involved an elderly, unmarried man who had migrated from England and had no relatives in Australia. Dr Nitschke recalled "*his sadness over the man's loneliness and isolation as he administered euthanasia*". Dr Nitschke has since revealed in testimony to a Senate committee, that he personally paid for this psychiatric consultation and that it in fact took less than 20 minutes.[41]

Dr David Kissane, comments on the issue of demoralisation:

Review of these patients' stories highlighted for me the importance of demoralization as a significant mental state influencing the choices these patients made. They described the pointlessness of their lives, a loss of any worthwhile hope and meaning.

Their thoughts followed a typical pattern of thinking that appeared to be based on pessimism, sometimes exaggeration of their circumstances, all-or-nothing thinking in which only extremes could be thought about, negative self-labelling and they perceived themselves to be trapped in this predicament. Often socially isolated, their hopelessness led to a desire to die, sometimes as a harbinger of depression, but not always with development of a clinical depressive disorder. It is likely that the mental state of demoralization influenced their judgement, narrowing their perspective of available options and choices. Furthermore, demoralized patients may not make a truly informed decision in giving medical consent.

Demoralization syndrome … is an important diagnosis to be made and actively treated during advanced cancer. It is recognised by the core phenomenology of hopelessness or meaninglessness about life. The prognostic language within oncology that designates 'there is no cure' is one potential cause of demoralization in these patients, a cause that can be avoided by more sensitive medical communication with the seriously ill. While truth telling is needed, hope must also be sustained so that life may be lived out as fully as possible. Patients with advanced cancer can be guided to focus on 'being' rather than 'doing', savouring the experiential moment of the present, so that purpose and meaning are preserved through inherent regard for the dignity of the person. Active treatment of a demoralized state by hospice services would involve counselling and a range of

[41] https://www.aph.gov.au/~/media/wopapub/senate/senate/commttee/S10740_pdf.ashx

complementary therapies, use of community volunteers and family supports, all designed to counter isolation and restore meaning.[42]

Conclusion

There is no model from any jurisdiction that has legalised assisted suicide and/or euthanasia for adequately ensuring that no person who is assisted to commit suicide or killed directly by euthanasia is suffering from treatable clinical depression or other forms of mental illness that may affect the capacity to form a competent, settled, determination to die by assisted suicide or euthanasia.

Jurisdictions, like Oregon, that provide for optional referral for psychiatric assessment manifestly fail to identify all cases of clinical depression.

The only jurisdiction which has required a psychiatric assessment for each case of euthanasia was the Northern Territory. However, this system signally failed to adequately identify depression, demoralization or other psychiatric issues which may have been treatable in all four cases of persons killed by former doctor Philip Nitschke under the Rights of the Terminally Ill Act 1995 (NT).

Compulsory referral to a psychiatrist, who may have never seen the person before, for a single brief assessment of whether the person's decision making capacity about assisted suicide or euthanasia is affected by depression or other psychiatric factors is clearly an inadequate safeguard and will not make assisted suicide "safe".

42 https://www.aph.gov.au/~/media/wopapub/senate/committee/legcon_ctte/completed_inquiries/2008_10/terminally_ill/submissions/sub589_pdf.ashx

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

BETTER OFF DEAD

Are the disabled at risk of wrongful death from an assisted suicide law?

- In the Netherlands and Belgium disability – both physical and intellectual – is accepted as a reason to euthanase a person.
- In Oregon the **five main reasons** given for requesting assisted suicide **all relate to disability** issues.
- People with disabilities are often seen in our society as "*better off dead*".
- Depression is less likely to be diagnosed and treated in a person with disability.
- The boundary between physical disability and terminal illness is fuzzy, not clear.

Netherlands and Belgium

A 2018 paper examines nine case reports on euthanasia in the Netherlands between 2012 and 2016 of people with an intellectual disability or an autism spectrum disorder.[43]

The case reports make for chilling reading, illustrating how once euthanasia becomes normalised in a society it becomes the go-to, accepted, "*final solution*" for "*difficult*" patients.

A man in his 60s with Asperger's, described as "*an utterly lonely man whose life had been a failure*", was euthanased because he was "horrified at moving into sheltered accommodation". Although he had been diagnosed with "*severe and probably chronic depression with a persistent death wish*" another psychiatrist, after seeing him just once, certified that he was free of depression in order to facilitate his euthanasia.

Another man in his 30s, also with Asperger's, was euthanased based on his distress at "*his continuous yearning for meaningful relationships and his repeated frustrations in this area, because of his inability to deal adequately with closeness and social contacts*".

A third case was of an intellectually disabled woman in her 60s who was euthanased for tinnitus despite a finding that:

the patient had indeed gone through many treatments in the past, but also, that often the wrong treatments had been instigated. It had also become clear to the physician that the

[43] https://bmcmedethics.biomedcentral.com/articles/10.1186/s12910-018-0257-6

patient often wanted to abandon the treatments, and that the treating practitioners had not encouraged her to try and persevere with these treatment(s) a bit longer.

In Case 2016-44 the Netherlands Review Committees approved the action of a doctor who euthanased a man aged between 80 and 90 years of age on the sole ground of having progressive loss of vision due to macular degeneration with his lack of capacity to read being accepted as unbearable and hopeless suffering.[44]

In Belgium, in December 2012 identical twin brothers were euthanased on the grounds of their psychological distress at learning they were both going blind. The brothers were reportedly distressed that they would not be able to see each other.[45]

Commenting on this case, Dr. Marc Maurer, President of the [US] National Federation of the Blind,[46] said:

"*This disturbing news from Belgium is a stark example of the common, and in this case tragic, misunderstanding of disability and its consequences. Adjustment to any disability is difficult, and deaf-blind people face their own particular challenges, but from at least the time of Helen Keller it has been known that these challenges can be met, and the technology and services available today have vastly improved prospects for the deaf-blind and others with disabilities. That these men wanted to die is tragic; that the state sanctioned and aided their suicide is frightening*."[47]

Disability issues accepted as leading reasons for assisted suicide: "You would be better off dead!"

Assisted suicide proposals usually would require two doctors to agree to a person's request for a prescription for a lethal dose to be used to end the person's life. To do so the doctors essentially need to agree that the person would "be better off dead" or at least that it is reasonable for a person in that position to consider that he or she would be better off dead.

The five main reasons given for requesting assisted suicide in Oregon in 2017 are **not primarily to do with pain (only 21%)** but rather with concerns about decreasing ability to participate in activities that made life enjoyable (88.1%), loss of autonomy (87.4%), loss of dignity (67.1%), physical or emotional burden on family, friends, or caregivers

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[44] https://www.euthanasiecommissie.nl/binaries/euthanasiecommissie/documenten/jaarverslagen/2016/april/12/jaarverslag-2016/RTE_jaarverslag2016.pdf

[45] http://www.telegraph.co.uk/news/worldnews/europe/belgium/9798778/Belgian-identical-twins-in-unique-mercy-killing.html

[46] http://www.facebook.com/NationalFederationoftheBlind

[47] https://nfb.org/national-federation-blind-comments-belgian-euthanasia-deaf-men-losing-sight

(55.2%) and the loss of control of bodily functions, such as incontinence and vomiting (37.1%) .[48] **These are all disability issues.**

Any assisted suicide proposal that includes a subjective notion of suffering as part of the eligibility criteria would allow assisted suicide for a similar set of concerns.

This set of concerns reflects the day to day realities of life for many people living with disabilities of various kinds.

If we legalise assisted suicide for incontinence, a loss of ability to engage in one's favourite hobby, a need to have other's take care of your physical needs, a loss of mobility and so forth what is the take home message for those who live with these challenges every day?

Are we saying they would also be better off dead?

Stella Young

The late Stella Young, comedian, writer and disability activist, wrote on the implications of legalising assisted suicide for people living with disabilities:



As a disabled person, I'm accustomed to conversations about quality of life and dignity. Specifically, I'm accustomed to assuring people that my life is worth living. I'm short statured, a wheelchair user, and I frequently have bone fractures. All the visual cues that make me 'the other' are front and centre.

People make all sorts of assumptions about the quality of my life and my levels of independence. They're almost always wrong.

I've lost count of the number of times I've been told, "I just don't think I could live like you," or "I wouldn't have the courage in your situation," or, ***my favourite one to overhear (and I've overheard it more than once), "You'd just bloody top yourself, wouldn't you?****". What we as a society think we know about what it means to live as a disabled person comes from cultural representations of disability seen through a nondisabled lens. And we, as people with disability, rarely get to tell our own stories.*

48 https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year20.pdf

Also, social attitudes towards disabled people come from a medical profession that takes a deficit view of disability. This is my major concern with legalising assisted death; that it will give doctors more control over our lives. As a disabled person who has had a lot to do with the medical profession, I can tell you that this is the space in which I've experienced some of the very worst disability prejudice and discrimination. Doctors might know about our biology, but it doesn't mean they know about our lives.

Media reports on assisted dying feed these misconceptions. ABC News reported this week on the case of Barbara Harling, a Queensland woman with motor neurone disease who said that she would consider moving to Tasmania if the Voluntary Assisted Dying Bill had passed. Harling is quoted as saying: "Well, let's put it this way. I can use my left hand, my right hand is just about useless. If I can't use my left hand to wipe my bottom, then I can do nothing else for myself. That means someone has to do everything for me. I couldn't bear to live like that."

The thing is, a lot of people do live like that. I know many, many people who depend on personal assistants for all of their daily living tasks, some of them requiring 24 hour care. ***Having to rely on someone else to wipe your bum may not be something anyone aspires to, but I'm quite sure it's never killed anyone.***

Perhaps our discomfort with this kind of thing is why we don't hear the counter view in reports about assisted dying. Often we hear supporters of euthanasia and assisted suicide talk about wanting to avoid the pain and suffering that often comes with imminent death. But more often, we hear stories like Barbara Harling's, which are more about wanting to avoid a loss of autonomy and independence. [49]

Screening for depression and coercion

Proposals for assisted suicide offer require assessing doctors to screen for depression or other mental health conditions that may be affecting the person's decision making capacity. Similarly the assessing doctors are supposed to determine that the request is voluntary and not the result of coercion.

Doctors are less likely to identify depression in people with disability, simply by assuming that it is normal for a person with disability to show signs of depression such as sadness and lack of hope.

Doctors may also easily miss the particular vulnerability of a person living with disability to overt or subtle coercion from family or caregivers who reinforce a feeling that the person is a burden, "*too much trouble*", "*life is too hard*" and so forth.

[49] http://www.abc.net.au/rampup/articles/2013/10/18/3872088.htm

Terminal condition or disability?

Proposals for assisted suicide may suggest making assisted suicide available to people with a specified length of time to live, such as six or twelve months or use some vaguer term such as where death is reasonably foreseen.

However, such proposals generally indicate that the prognosis should take account of which treatments are "acceptable to the person". On this basis various condition that are not in themselves terminal may be considered as terminal for the purpose of offering assisted suicide if the person decides to forego an effective, available treatment for whatever reason.

This approach poses a severe risk to people following an initial acquisition or diagnosis of a condition that may involve a considerable level of disability.

Marilyn Golden writes :

Research overwhelmingly shows that people with new disabilities frequently go through initial despondency and suicidal feelings, but later adapt well and find great satisfaction in our lives.

However, the adaptation usually takes considerably longer than the mere fifteen-day waiting period [generally] required by assisted suicide proposals. People with new diagnoses of terminal illness appear to go through similar stages. In that early period before one learns the truth about how good one's quality of life can be, it would be all too easy, if assisted suicide is legal, to make the final choice, one that is irrevocable.

Dr. Richard Radtke, a well-known retired academic oceanographer in Hawaii, provides one such example. Dr. Radtke has had a very disabling form of muscular sclerosis for over 25 years. In the period after his diagnosis, doctors often classified him as terminally ill. He experienced severe depression for two years. Had assisted suicide been legal, he acknowledges that he would have chosen it and died long ago. Today, still with an extremely limiting disability, he has retired from a successful academic career, is a happily married father, remains the president of a charitable foundation, and is grateful for the length and varied experiences of his life.

How many such individuals is our society prepared to sacrifice as the collateral damage from the legalization of assisted suicide? [50]

Conclusion

Legalising assisted suicide poses a direct threat to the lives of some people with disabilities who may be assessed as eligible to request it. Doctors are more likely to agree that they are "*better off dead*" and to miss signs of depression or coercion.

[50] https://dredf.org/public-policy/assisted-suicide/why-assisted-suicide-must-not-be-legalized/

Legalising assisted suicide for being a burden, incontinence and loss of ability to enjoy activities trivialises issues faced daily by persons living with disability and demeans their courage in facing the challenges of life.

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

BULLYING OR COERCION

Can we rule out coercion if we legalise assisted suicide?

- Assisted suicide and euthanasia laws usually require that a request be voluntary and free of coercion. To be truly voluntary a request would need to be not just free of overt coercion but also **free from undue influence, subtle pressures and familial or societal expectations**.
- A regime in which assisted suicide is made legal and in which the decision to ask for assisted suicide is positively affirmed as a wise choice in itself creates a framework in which a person with low self-esteem or who is more susceptible to the influence of others may well express a request for assisted suicide that the person would otherwise never have considered.
- Elder abuse, including from adult children with "***inheritance impatience***" is a growing problem in Australia. This makes legalising assisting suicide unsafe for the elderly.
- Evidence from jurisdictions that have legalised assisted suicide or euthanasia shows that coercion, including the feeling of **being a burden on others, is a real problem.**
- Some supporters of assisted suicide **don't care if some people are bullied into killing themselves** under an assisted suicide law.

Oregon and Washington

The data from Oregon shows that in 2018 more one out of two (54.16%) people who died after taking prescribed lethal medication cited concerns about being a "*Burden on family, friends/caregivers*" as a reason for the request.[51]

In a further 14.9% of cases (nearly one in seven cases) the attending physician reported not knowing if the person who requested lethal medication and subsequently died after

[51] Oregon Public Health Division, *Oregon Death With Dignity Act: 2018 Data Summary, Table 1,* p.12 https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year21.pdf

ingesting had a concern about physical or emotional burden on family, friends or caregivers.[52]

ORS 127.815 sets out as the very first responsibility of an attending physician under the Death With Dignity Act a duty to "*Make the initial determination of whether a patient has a terminal disease, is capable, and has made the request voluntarily*"

How can a physician come to a firm conclusion that a person is voluntarily requesting lethal medication in order to end their lives without exploring whether or not the person is motivated by a concern about the physical or emotional burden on family, friends or caregivers.

Surely such a discussion is necessary to exclude any possibility that the person is making the request under duress, subject to coercion or undue influence from a family member or caregiver.

Additionally, in the absence of such a discussion there may be a missed opportunity to relieve the person's concern about being a burden by arranging respite for family caregivers or additional care or support.

If the 14.9% of cases where the attending physician does not even bother exploring this issue with a person before writing a prescription for lethal medication are added to the 54.2% of cases in 2018 where the attending physician reports knowing that the person had a concern about the physical or emotional burden on family, friends or caregivers then in nearly seven out of ten cases (69.1%) concern about being a burden is or maybe a factor in a request for lethal medication.

The data from Washington[53] shows that in 2017 more than half (56%) of those who died from prescribed lethal drugs cited concerns about being a "*Burden on family, friends/caregivers*" as a reason for the request.

Does the concern about being a burden originate from the person or is it generated by subtle or not so subtle messages from family, friends and caregivers - including physicians - who find the person to be a burden or a nuisance or just taking too long to die?

Elder law expert Margaret Dore comments:

In both Washington and Oregon, the official reporting forms include a check-the-box question with seven possible "concerns" that contributed to the lethal dose request. These concerns include the patient's feeling that he was a "burden."

The prescribing doctor is instructed: "Please check 'yes,' 'no,' or 'don't know' depending on whether or not you believe that a concern contributed to the request."

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52 https://assets.nationbuilder.com/australiancarealliance/pages/96/attachments/original/1552018763/Oregon_-_Twenty_one_years_of_assisted_suicide.pdf?1552018763

53 https://www.doh.wa.gov/Portals/1/Documents/Pubs/422-109-DeathWithDignityAct2017.pdf

In other states, a person being described as a "burden" is a warning sign of abuse.

For example, Sarah Scott of Idaho Adult Protection Services describes the following "warning sign":

"Suspect behavior by the caregiver . . . [d]escribes the vulnerable adult as a burden or nuisance."

The recommendation is that when such "warning signs" exist, a report should be made to law enforcement and/or to the local adult protective services provider.

Washington and Oregon, by contrast, instruct its doctors to check a "burden" box. Washington and Oregon promote the idea that its citizens are burdens, which justifies the prescription of lethal drugs to kill them. Washington's and Oregon's Acts do not promote patient "control," but officially sanctioned abuse of vulnerable adults.[54]

Belgium

Claire-Marie Le Huu-Etchecopar is a French nurse who has worked in Belgium since 2008. She has written about her experience with euthanasia in Belgium.

She describes the actions of the friends of a patient:

> *Those close to her are locked in the emotion of seeing their friend disabled. They cannot bear to see her different. Any other solution than euthanasia seems unimaginable to them. In a small notebook where they leave her messages while she's sleeping, the question of euthanasia is on every page. You can read words such as:*
>
> > *Do not forget your euthanasia, it is your right, you have to ask the doctors or they'll never do it for you...* [55]

Elder abuse

Undue influence is increasingly being seen as a relevant factor in the financial abuse of elders.

Seniors Rights Victoria provides a useful summary of case law and best practice on undue influence in the financial abuse of elders.[56]

It is clear from this summary that undue influence can easily be missed and may be difficult to identify. Of course, the courts can apply the remedy of recission if it is

[54] http://scholarship.law.marquette.edu/elders/vol11/iss2/8

[55] http://alexschadenberg.blogspot.com.au/2014/05/lifting-veil-on-euthanasia-what-really.html

[56] https://assetsforcare.seniorsrights.org.au/relationship-breaks-down/equity/undue-influence-unconscionable-dealing/

established. In the case of assisted suicide a failure to spot undue influence before writing a prescription for a lethal dose will be incapable of remedy once the lethal dose is ingested.

A recent parliamentary report on Elder Abuse in New South Wales also referenced the failure of professionals to identify undue influence and so unwittingly facilitate elder abuse.[57]

It cites the Council on the Ageing NSW as observing that the *NSW Interagency policy on preventing and responding to abuse of older people:*

> *does not address the more common cases where elder abuse is perpetrated by a family member or carer 'in an environment of isolation, dependence and undue influence*. (para 5.13 on p. 54)

The report also notes that:

> *Capacity Australia observed that financial abuse is often fueled by ignorance and family conflict, as well as 'inheritance impatience'. It further noted that undue influence by one family member over another is commonly facilitated by legal professionals because of their failure to detect when an older person is struggling to manage their financial affairs, that is, when they lack financial capacity*. (para 6.6 on p. 80)

As long ago as 1885 in what is still cited in Australian law as the leading case on undue influence, Sir James Hannen described some of the kinds of subtle coercion that a frail, elderly or ill person may be subjected to that could be hard for any outside person to detect.

The coercion may of course be of different kinds, it may be in the grossest form, such as actual confinement or violence, or a person in the last days or hours of life may have become so weak and feeble, that a very little pressure will be sufficient to bring about the desired result, and it may even be that ***the mere talking to him at that stage of illness and pressing something upon him may so fatigue the brain, that the sick person may be induced, for quietness' sake, to do anything.*** *This would equally be coercion, though not actual violence.* [58]

If a few grannies are bullied …

Dr Henry Marsh, a British neurosurgeon and proponent of legalising assisted suicide and euthanasia, has acknowledged the possibility of coercion and elder abuse leading to wrongful deaths under such a law but he simply doesn't care:

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[57] https://www.parliament.nsw.gov.au/committees/DBAssets/InquiryReport/ReportAcrobat/6063/Report%252044%2520-%2520Elder%2520abuse%2520in%2520New%2520South%2520Wales.pdf

[58] https://swarb.co.uk/wingrove-v-wingrove-1885/

"Even if a few grannies get bullied into [suicide], isn't that the price worth paying for all the people who could die with dignity?" [59]

Conclusion

It is clear from this evidence that simply requiring a physician to tick a box stating the person requesting assisted suicide is doing so voluntarily is no guarantee that the physician has the competence or has undertaken the extensive and careful inquiries necessary to establish that the person is not subject to undue influence or subtle pressure (albeit unwittingly) from family, friends or society to request assisted suicide so as not to burden others.

No jurisdiction that has legalised assisted suicide has even made any serious effort to establish a genuinely safe framework in this regard. Indeed no such framework is possible.

Any law permitting assisted suicide or euthanasia will inevitably result in wrongful deaths from coercion.

[59] https://www.medscape.com/viewarticle/879187

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

SOCIAL CONTAGION OF SUICIDE

Does legalising assisted suicide affect the suicide rate?

- Legalising assisted suicide for some Australians undermines the commitment to **suicide prevention for all** Australians
- Legalising assisted suicide has been shown to lead to **an increase** in the overall rate of suicides of 6.3% and of the elderly (65 years and older) by 14.5%
- This is outcome is predictable because of the well-known **Werther effect of suicide contagion whenever suicide is presented in a positive light** as a romantic or rational act
- The families of those who commit suicide under an assisted suicide law suffer high rates of posttraumatic stress disorder

Should suicide prevention strategies include all Australians?

Proposals to legalise assisted suicide or euthanasia for a select group of people, such as the terminally ill, necessarily imply that society agrees such people may be better off dead and supports their suicide as a legitimate, rational choice to be facilitated rather than prevented.

The question arises as to how publicly and openly offering assistance to commit suicide to one group of Australians fits with the public policy goal, widely shared across the whole community, to reduce the incidence of suicide?

Does legalising assisted suicide reduce the suicide rate as claimed?

Proponents of assisted suicide have claimed that providing the elderly, terminally ill with a legal lethal dose of drugs to facilitate assisted suicide will reduce the incidence of other forms of suicide among this group and, because, it is claimed, many of those for whom the lethal dose is prescribed may never take it, actually decrease the overall suicide rate.

This hypothesis has been subjected to careful scrutiny in an important study by David Albert Jones and David Paton comparing trends in suicide rates in those states of the United States which have legalised assisted suicide compared to those which have not.[60]

[60] https://pdfs.semanticscholar.org/6df3/55333ceecc41b361da6dc996d90a17b96e9c.pdf

The study, which controlled for various socio-economic factors, unobservable state- and year effects, and state-specific linear trends, found that legalizing assisted suicide was associated with a 6.3% increase in total suicides (i.e. including assisted suicides).

This effect was significantly larger in the over 65 year old age group with a massive 14.5% increase in total suicides.

The introduction of legalised assisted suicide was not associated with a reduction in non-assisted suicide rates, nor with an increase in the mean age of non-assisted suicide.

The conclusion is that assisted suicide either does not inhibit (nor acts as an alternative to) non-assisted suicide, or that it acts in this way in some individuals but is associated with an increased inclination to suicide in other individuals.

The latter suggestion would be consistent with the well known Werther effect of suicide contagion.[61]

Victoria

This conclusion is supported by **evidence from Victoria**.

When arguing for the legalisation of State-approved and funded assistance to suicide, then Minister for Health and Human Services, the Hon Jill Hennessy, claimed that:

Evidence from the coroner indicated that one terminally ill Victorian was taking their life each week.[62]

The *Voluntary Assisted Dying Act 2017*, which she introduced on behalf of the Victorian Government, excluded deaths by self-administration of a "voluntary assisted dying substance" for the purpose of causing a person's death from being considered as caused by suicide.

By this legal fiction, which is replicated in Section 8 of the Bill, such deaths are recorded as caused by the disease, illness or medical condition cited by a doctor in the application for a self-administration permit under the Victorian Act.

If Ms Hennessy's claim was correct there ought to have been **a decrease of around 50 deaths by suicide each year** once the Act came into operation, as it did on 19 June 2019, as these terminally ill Victorians would now have access to a State-approved and State-funded way to intentionally cause their own deaths by ingesting a lethal poison.

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[61] http://journals.sagepub.com/doi/abs/10.1080/00048670701266680

[62] https://www.parliament.vic.gov.au/images/stories/daily-hansard/Assembly_2017/Assembly_Daily_Extract_Thursday_21_September_2017_from_Book_12.pdf

However, according to the Coroners Court of Victoria there were 694 deaths by suicide in Victoria in 2017.[63]

There were slightly more - 698 - in 2020, which was the first full calendar year in which State issued suicide permits and the State-funded poison delivery service were in operation.

So, there is no evidence that the anticipated decrease of 50 deaths by (non-authorised) suicide each year has been achieved.

Moreover, if we put aside the legal fiction declaring suicides pursuant to a permit issued by the Victorian Secretary of the Department of Health and Human Services there were an additional 144 suicides in 2020 which were officially recorded by the Voluntary Assisted Dying Review Board as "Confirmed deaths - Medication [sic] was self-administered".

Adding these 144 State-approved, State-funded suicides by the ingestion of State-supplied lethal poison to the 698 suicides without such State approval and facilitation gives a total of 842 suicides in 2020 - an increase of 21.2% on 2017.

The 144 suicides with State approval in 2020 are nearly three times the 50 suicides of terminally ill persons each year claimed by Minister Hennessy during the 2017 parliamentary debate.

Additionally, 31 Victorians were killed by injection of State-funded and supplied lethal poisons by a doctor who had been issued, a voluntary assisted dying physician administered permit, by the Secretary of the Department for Health and Human Services, specifically **authorising the doctor to administer the poisons in order to cause the death of the person**.

If these are added to the count of suicides - insofar as they are at least purported to be **performed at the request of the person with the intention of causing that person's death** - then the total for 2020 would be 873 - a 25.8% rise since 2017.

Effect on families

Suicide is a distressing event that disrupts the lives of families, friends and communities who are bereaved.

Like any other suicide, assisted suicide can profoundly affect surviving family members and friends.

[63] https://www.coronerscourt.vic.gov.au/sites/default/files/2021-01/Coroners%20Court%20Monthly%20Suicide%20Data%20Report%20-%20December%202020.pdf

A recent study found that about 20% of family members or friends who witnessed an assisted suicide in Switzerland, where assisted suicide is legal, subsequently suffered from full (13%) post-traumatic stress disorder or subthreshold (6.5%) post-traumatic stress disorder. [64]

Conclusion

Proposals to promote assisted suicide for some people runs an unacceptable risk of undermining efforts to prevent suicide for all other members of the community and of increasing the trauma suffered by families, friends and communities due to the suicide of loved ones.

[64] https://linkinghub.elsevier.com/retrieve/pii/S0924933810002683

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

KILLED WITHOUT REQUEST (OR WHILE RESISTING)

Those who are killed **without any request** by doctors who have grown used to the practice of ending their patients' lives are clearly wrongful deaths. In some cases a doctor has performed euthanasia even where a person resisted.

Netherlands

In 2015 there were 431 cases of euthanasia without explicit request, representing 6.06% (or more than one out of sixteen) of all euthanasia deaths.[65]

More than 1 in 200 (0.52%) of all deaths (other than sudden and expected deaths) of 17-65 year olds in the Netherlands are caused intentionally by euthanasia without an explicit request from the person being killed.

In Case 2016-85 the Review Committees found that a doctor had not acted with due diligence in administering euthanasia to a woman with Alzheimer's disease. The woman had made a general reference in a living will to wanting euthanasia at the "right time". At the time the doctor euthanased her she was incompetent to voluntarily request it.

The doctor put medication in her coffee to reduce her consciousness deliberately so as to avoid her resisting being given drugs. Nonetheless she physically struggled against the administration of an intravenous lethal injection. She was physically restrained by family members while the doctor completed the administration of the lethal drugs.[66]

On 13 June 2018 the Regional Disciplinary Court for Healthcare in The Hague considered a complaint against the doctor brought by the Inspectorate for Health Care and Youth. The decision was published on 24 July 2018.[67]

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[65] https://opendata.cbs.nl/statline/#/CBS/en/dataset/81655ENG/table?ts=1525401083207

[66] https://www.euthanasiecommissie.nl/binaries/euthanasiecommissie/documenten/jaarverslagen/2016/april/12/jaarverslag-2016/RTE_jaarverslag2016.pdf

[67] https://www.tuchtcollege-gezondheidszorg.nl/binaries/tuchtcolleges-gezondheidszorg/documenten/publicaties/documentatie-procedures/uitspraken/uitspraken-van-persberichten/beslissing-euthanasie-bij-dementie/2018-033bes.pdf

The Court found that the written declaration of intent was not sufficiently clear to justify euthanasia in this case. It also found that the doctor should have tried to discuss the execution of euthanasia with the patient beforehand.

> *In view of the irreversibility of termination of life and the ethical aspects connected with the deliberate ending of the life of a fellow human being, a written euthanasia declaration must be unambiguous, not needing any further interpretation.*

The Court did not completely ruled out that ambiguities in a written declaration of intent could be removed (even in the case of a demented patient) if a patient is later unambiguous, consistent and tenacious (verbally or non-verbally) in his statements about wanting death. However, with this patient this was not the case because she sometimes said she wanted to die and sometimes not.

Despite its finding that the doctor had seriously breached the requirements for euthanasia it only imposed a reprimand on the doctor.

On 9 November 2018 it was announced that a criminal investigation into this case by the Board of Public Prosecutors had concluded and that the doctor would be prosecuted.[68]

This is the first time that the Dutch Public Prosecution Service (OM) will prosecute a doctor for euthanasia since the introduction of the Act on Termination of Life on Request and Assisted Suicide in 2002.

> *After extensive investigation, the public prosecutor came to the conclusion that the nursing home doctor had not acted in accordance with the legal standards. The public prosecutor considers it important* **that the court assesses whether the doctor was entitled to rely on the living will completed by the woman**. *In addition, the OM reproaches the physician that she assumed that the woman still wanted to die without verifying this with the woman. Although the woman had regularly stated that she wanted to die, on other occasions she had said that she did not to want to die. In the opinion of the OM,* **the doctor should have checked with the woman whether she still had a death wish by discussing this with her**. *The fact that she had become demented does not alter this, because according to the Public Prosecution, the law also requires the doctor to verify the euthanasia request in such a situation.* **These two legal questions on the termination of life of people suffering from dementia justify the submission of this case to the criminal court judge**.

It is not yet known when the case will be heard by the District Court of The Hague.

[68] https://www.om.nl/vaste-onderdelen/zoeken/@104443/nursing-home-doctor/

WRONGFUL DEATHS BY ASSISTED SUICIDE OR EUTHANASIA

NOT A RAPID OR PEACEFUL DEATH

The case for legalising assisted suicide and euthanasia **simplistically assumes** that once legalised such deaths will be both rapid and peaceful. However, this is not the case. As a recent article in the journal *Anaesthesia* found:

> *Complications related to assisted dying methods were found to include difficulty in swallowing the prescribed dose (≤9%), a relatively high incidence of vomiting (≤10%), prolongation of death (by as much as seven days in ≤4%), and failure to induce coma, where patients re-awoke and even sat up (≤1.3%).*
>
> *This raises a concern that some deaths may be inhumane.*

Netherlands

Technical problems, complications and problems with completion in the administration of lethal drugs for euthanasia have been reported from the Netherlands.

Technical problems occurred in 5% of cases. The most common technical problems were difficulty finding a vein in which to inject the drug and difficulty administering an oral medication.

Complications occurred in 3% of cases of euthanasia, including spasm or myoclonus (muscular twitching), cyanosis (blue colouring of the skin), nausea or vomiting, tachycardia (rapid heartbeat), excessive production of mucus, hiccups, perspiration, and extreme gasping. In one case the patient's eyes remained open, and in another case, the patient sat up.

In 10% of cases the person took longer than expected to die (median 3 hours) with one person taking up to 7 days.

From 2016 to July 2018 the Board of Procurators General reported on 11 cases of euthanasia with serious breach of protocols by the doctor, including:

- a failed assisted suicide because the doctor ordered the wrong drug;
- seven cases of the muscle relaxant being administered when the person was not in a full coma and therefore **potentially causing pain**; and
- three cases where a first attempt at euthanasia failed and the doctor had to leave the person to get a second batch of lethal drugs.

Oregon

In Oregon in 2020 nearly all (99.6%) deaths by ingesting a lethal dose involved an experimental cocktail in which morphine sulphate was the main lethal substance. These experimental lethal cocktails do not always result in a swift and peaceful death.
In fact, there are reported complications each year, with an **overall failure rate of 0.42%** (8 people recovered consciousness out of 1905) and an **overall complication rate of 6.3%** (52 out of 827 people for whom this data is available).

In 2020 there were five cases of complications out of 72 – 6.94% of those for whom information about the circumstances of their deaths is available. This included one case of **seizures** and 3 cases of difficulty ingesting or regurgitating the poison. In 2019 nearly **one in ten** (9.84%) experienced complications. In 2018 nearly **one in eight** (12.12%) had complications and additionally, one person failed to die and regained consciousness. Two people had seizures in 2017.

The **interval from ingestion of lethal drugs to unconsciousness has been as long as four hours** (in 2017). In 2019, at least one person took 90 minutes after ingestion to lose consciousness.

The **time from ingestion to death has been as long as 104 hours** (4 days and 8 hours) in a person who ingested **pentobarbital** (the lethal poison being used for suicides under Victoria's law).

One person in 2019 took nearly two days (47 hours) to die after using DDMP2 and another person took 19 hours to die after using DDMA In 2020 one person took 8 hours to die after using DDMA, and another two people took more than 6 hours to die.

8 people have regained consciousness after taking the supposedly lethal dose, including one person in 2018.

In 2005, one person became unconscious 25 minutes after ingestion, then regained consciousness 65 hours later.

This was lumberjack David Prueitt who, after ingesting the prescribed barbiturates spent three days in a deep coma, then suddenly woke up, asking his wife *"Honey, what the hell happened? Why am I not dead?*" David survived for another 14 days before dying naturally from his cancer. ["Oregon man wakes up after assisted-suicide attempt", *Seattle Times,* 4 March 2005]

Since 2005 seven other people have regained consciousness after ingesting the lethal medication.

"*In 2010, two patients regained consciousness after ingesting medications. One patient regained consciousness 88 hours after ingesting the medication, subsequently dying from*

underlying illness three months later. The other patient regained consciousness within 24 hours, subsequently dying from underlying illness five days following ingestion.

In 2011, two patients regained consciousness after ingesting the medication. One of the patients very briefly regained consciousness after ingesting the prescribed medication and died from underlying illness about 30 hours later. The other patient regained consciousness approximately 14 hours after ingesting the medication and died from underlying illness about 38 hours later."

In 2012 "*one patient ingested the medication but regained consciousness before dying of underlying illness … The patient regained consciousness two days following ingestion, but* ***remained minimally responsive and died six days following ingestion***".

In 2017 "*one patient ingested the medication but regained consciousness before dying from the underlying illness*". In 2018 one person regained consciousness after ingesting the prescribed substance and later died of the underlying illness. These two occurred after using DDMP2 – one of the experimental lethal cocktails being used by pro-assisted suicide doctors.

Washington

In 2017 one person took 6 hours to lose consciousness after ingesting the lethal dose and one person took 35 hours to die after ingesting the lethal dose. In 2016 one person took 11 hours to lose consciousness after ingesting the lethal dose. In 2015 one person took 72 hours (3 days) to die after ingesting the dose. In 2013 one person took 3 hours to lose consciousness after ingesting the lethal dose and one person took 41 hours (1 day and 17 hours) to die after ingesting the dose. In 2009 two people awakened after initially losing consciousness. In 2014 one person suffered seizures after ingesting the lethal medication.

At least 18 patients have regurgitated the lethal medication. Seven of these cases occurred in 2016 alone.

This may be related to the use of new experimental cocktails of lethal drugs being used since the price of the previously used drugs, secobarbital and pentobarbital (Nembutal), escalated.

The first of the new cocktails is a mix of phenobarbital, chloral hydrate and morphine sulfate. It was prescribed in 88 cases in 2015 and 106 cases in 2016 but no longer prescribed in 2017 no doubt due to the fact that it was found to be very caustic and to cause a profound burning in the throat.

The second experimental cocktail includes morphine sulfate, propranolol, diazepam, digoxin and a buffer suspension. It was used in 4 cases in 2015, 53 cases in 2016 and 130 cases (66%) in 2017.

Death penalty

Although the legal reasons for intentionally ending a human life under the death penalty or by euthanasia or assistance to suicide are different, in all these cases **the same physiological process – causing death rapidly and painlessly – is the stated aim**.

It is not surprising then that some of the **same lethal substances** are used.

Sodium pentobarbital has been used for assistance to suicide in the Netherlands, Oregon and Washington (until 2015), and Victoria as well as by several States in the United States in the execution of prisoners.

David Waisel, MD, an anaesthesiologist, has testified about the use of this drug in executions:

> *… as the lethal injection commenced Mr. Blankenship jerked his head toward his left arm and made a startled face while blinking rapidly. He had a "tight" grimacing expression on his face and leaned backward. Shortly thereafter, Mr. Blankenship grimaced, gasped and lurched twice toward his right arm. During the next minute, Mr. Blankenship lifted his head, shuddered and mouthed words. Three (3) minutes after the injection, Mr. Blankenship had his eyes open and made swallowing motions. Four (4) minutes after injection, Mr. Blankenship became motionless. About thirteen (13) minutes after the injection, Mr. Blankenship was declared dead. Again, his eyes were open throughout.*
>
> *Based on his lurching toward his arms and the lifting of his head and the mouthing of words, I can say with certainty that Mr. Blankenship was inadequately anesthetized and was conscious for approximately the first three minutes of the execution and that he suffered greatly. Mr. Blankenship should not have been conscious or exhibiting these movements, nor should his eyes have been open, after the injection of pentobarbital.*
>
> *Given prior executions of Brandon Rhode and Emanuel Hammond in September 2010 and January 2011, respectively, during which these inmates reportedly exhibited similar movements and opened their eyes (Rhode's eyes were open throughout the execution process), Mr. Blankenship's execution further evidences that* ***during judicial lethal injections in Georgia there is a substantial risk of serious harm such that condemned inmates are significantly likely to face extreme, torturous and needless pain and suffering***. (State of Massachussetts, County of Suffolk., *Affidavit of David B. Waisel, MD*, p. 2-3)

Another anaesthesiologist, Joel Zivot, MD writes:

> *In 2014, I watched the lethal injection of Marcus Wellons in a Georgia prison.*

I noticed that Wellons's fingers were taped to the stretcher, which made little sense, given his body had already been restrained by heavy straps. I kept asking myself why. I read into the subject and came across a report of the lethal injection execution of another death row inmate, Dennis McGuire, five months earlier. During that 24-minute process at the Ohio jail, McGuire clenched his fists. Perhaps it was a final, futile show of defiance. Perhaps it was an outward display of pain. With his fingers secured, Wellons could not have made any such gesture.

In 2017, I obtained ***a series of autopsies of inmates executed by lethal injection****, which confirmed my worst fears. Wellons's autopsy revealed that his lungs were profoundly congested with fluid, meaning they were around twice the normal weight of healthy lungs. He had suffered what is known as pulmonary oedema, which could only have occurred as he lay dying.* ***Wellons had drowned in his secretions****. Yet even my medical eye detected no sign of distress at his execution.*

Wellons was executed with a chemical called pentobarbital, which caused his pulmonary oedema. If ***a post-mortem examination were to be performed on a body after assisted suicide, it's very likely that similar pulmonary oedema would be found****.*

A 2020 review, published by NPR, of 216 autopsies conducted after execution in US States by lethal injection found signs of pulmonary oedema in 84% of the cases. The findings were similar across the states and, notably, across the different drug protocols used.

> *Not all states conduct autopsies after lethal injections. Texas, which has performed by far the most lethal injections of any state, has a policy of not conducting autopsies. When asked by NPR about this, a spokesperson for the Texas Department of Criminal Justice said, "We know how they died."*

Euthanasia and assistance to suicide laws often include provisions designed to preclude autopsies such as excluding such deaths from being "reportable" to the Coroner and falsifying the death certificate by stating the cause of death as the medical condition for which euthanasia or assistance to suicide had been requested.

The protocol for self-administration of a lethal poison to cause death under Victoria's law, for example, includes an anti-emetic and an anti-anxiety drug to be taken before drinking a mixture containing 15 g of pentobarbital. However, the person will still be conscious when the pentobarbital is ingested.

Dr Zivot comments that "*without a general anaesthetic, many will be in great discomfort, even if outwardly they don't appear to be suffering*."

In her 2015 dissent in *Glossip v Gros*, US Supreme Court Justice Sotomayor, characterised death by lethal injection as *"the chemical equivalent of being burned at the stake".*

While drug regimes used to execute offenders in the United States are subject to scrutiny in the courts where evidence can be presented that they may cause a painful death, there is no such scrutiny of the drug regimes used for euthanasia and assistance to suicide, which are just as likely to result in such painful and inhuman deaths.

LACKING CAPACITY

Are persons lacking decision making capacity at risk of wrongful death by assisted suicide?

- A recent landmark study shows that the majority of persons diagnosed with a terminal illness and with less than 6 months to live lack full decision making capacity;
- Regimes that permit assisted suicide with no requirement for a doctor to be present when the lethal substance is ingested only require assessment of decision making capacity at the time of the request – not at the time it is ingested.

Decision making capacity and terminal illness

In a landmark study of decision making capacity of persons with terminal cancer and a prognosis of less than six months to live – that is a cohort that would be eligible for assisted suicide under the schemes in Oregon and other US States as well as in Victoria, Australia – 90% were found to be impaired in regard to at least one of the four elements of decision making – Choice (15% impaired), Understanding (44%), Appreciation (49%) and Reasoning (85%).[69]

Under Victoria's *Voluntary Assisted Dying Act 2017*, for example, "a person is presumed to have decision-making capacity unless there is evidence to the contrary" (Section 4(2)).

This study suggests that, at least in the case of persons with cancer and a prognosis of less than six months to live, it would be more prudent to start from the presumption that they are likely to have impaired decision making capacity unless it is demonstrated to the contrary.

The study also found a significant discrepancy between physician assessments of decision making capacity compared to the actual decision making capacity as tested on the MacCAT-T scales.

Physicians assessed as "unimpaired" 64% of those who, according to the MacCAT-T assessment had impaired Reasoning; 70% who had impaired Appreciation; 61% who had impaired Understanding and 100% of those who had impaired Choice.

[69] Elissa Kolva et al., "Assessing the decision making capacity of terminally ill patients with cancer", *American Journal of Geriatric Psychiatry,* 2018 May; 26(5): 523–531, https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6345171/pdf/nihms937741.pdf

This lack of ability of physicians who are actually caring for terminally ill cancer patients with a prognosis of less than six months to live to accurately assess their patients' decision making capacity is likely to be exceeded in flawed assessments of decision making capacity by other doctors – who do not necessarily have an established relationship with the person – making an assessment of decision making capacity in relation to a request for assisted suicide.

Schemes like those in Oregon and Victoria provide for assessing doctors to refer to a specialist for an assessment of decision making capacity but this is **purely optional** on the part of those doctors. In Oregon in 2018 only 3 out of 168 people (1.78%) who died by assisted suicide under the Oregon law were referred by the prescribing doctor to a psychiatrist or psychologist for consultation on whether or not the person was not "capable" due to "impaired judgement".[70]

Loss of decision making capacity between assessment at time of request and time of ingestion

Oregon and Washington report data on the length of time between a person requesting a lethal substance for the purpose of assisted suicide and actually ingesting the substance.

In Oregon in 2018 one person ingested lethal medication 807 days (2 years 2 ½ months) after the initial request for the lethal prescription was made. The longest duration between initial request and ingestion recorded is 1009 days (that is 2 years and 9 months).[71] In Washington the data shows that in each year between 5% and 17% of those who die after requesting a lethal dose do so more than 25 weeks later with one person in 2012 dying nearly 3 years (150 weeks) later, one person in 2015 dying nearly two years later (95 weeks) and one person in 2016 dying more than two years (112 weeks) later.[72]

Clearly in these cases there is a real possibility that, even if a person was correctly assessed as having decision making capacity at the time of the request they may well

[70] Oregon Public Health Division, *Oregon Death With Dignity Act: 2018 Data Summary, Table 1,* p.11, https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year21.pdf

[71] Oregon Public Health Division, *Oregon Death With Dignity Act: 2018 Data Summary, Table 1,* p.13, https://www.oregon.gov/oha/PH/PROVIDERPARTNERRESOURCES/EVALUATIONRESEARCH/DEATHWITHDIGNITYACT/Documents/year21.pdf

[72] Washington State Department of Health, *Death with Dignity Act Reports,* 2009-2017 available at: http://www.doh.wa.gov/YouandYourFamily/IllnessandDisease/DeathwithDignityAct/DeathwithDignityData

have since become impaired in their decision making capacity before actually ingesting the lethal substance.

Under their schemes (and under similar schemes in other US States as well as Victoria, Australia) the only assessment of decision making capacity is carried out at the time of the request. There is no assessment of decision making capacity at the time the lethal substance is actually ingested.

This obviously creates the opportunity for subtle or overt coercion, or for deceptive administration of the lethal substance by a family member or carer.[73] However, it also makes it possible for the person to voluntarily ingest the lethal substance but without a full appreciation or understanding of what they are doing and without making a reasoned decision to do so.

No scheme can avoid wrongful deaths due to lack of capacity

Given that, even if doctors assessing decision making capacity improved their skills beyond the present very poor level, there will still be persons who are mistakenly assessed as having decision making capacity who actually are impaired in their ability to understand, appreciate or make a reasoned decision about assisted suicide or euthanasia, there will inevitable be wrongful deaths from lack of capacity.

Additionally, in those jurisdictions which allow persons requesting a lethal substance for assisted suicide to be prescribed and supplied with the lethal substance for later ingestion there is a very real possibility that some of these people will have impaired decision making capacity by the time (perhaps weeks, months or even years later) when they actually ingest it. These too will die a wrongful death.

[73] https://www.australiancarealliance.org.au/bullying_or_coercion